

Patient Monitoring System (PMS)

Part 1: Feedback on your report

Goddé-Ramaekers-VanVyve

This report will provide you with some concrete feedback on different aspects of your part 1 report. The colored frame below shows your overall score for part 1. This takes into account all the aspects such as ASR coverage, quality of description, quality of business value and architectural impact, quality of your QASS, concreteness, responses, suitability of metrics, etc. To help you understand and interpret this score, this report provides more detailed and concrete feedback on each of those aspects.

68% – 20.4/30

General Comments Overall, your submission is of satisfactory quality. However, there is still potential left on the table when it comes to deeper reasoning, specificity, and attention to detail. Many of your ASRs (in particular, motivations for assigned business value and architectural impact) stay too brief and high-level. **Specific attention points**

- When explaining a business value or architectural impact, be careful not to just repeat the requirement again, but explain *why* it is important. For example, in ASR 15 you say ‘crucial for billing’, but you do not explain why this is crucial for billing, or what the business impact would be if the requirement is not met.
- Although brevity and being to-the-point is appreciated, this should not come at the cost of important details. Many of your ASRs have (too) short (and vague) business value and/or architectural impact motivations. For example: ASR 2, ASR 13, ASR 15, ...
- When explaining architectural impact, you sometimes provide a few requirements that need to be met in the architecture to realize the ASR. However, be aware that a necessary requirement is not always a sufficient requirement. Quite a few times, the requirements in architectural impact you list are not comprehensive/complete to achieve the desired quality requirement. One example is ASR 13: you mention the data schema to support the requirement, but you do not talk about interfaces in which physicians can enter new configurations, mechanisms to sync desired configuration of devices with the backend, etc.
- For architectural impact, be careful not to simply repeat the ASR, but explain how this could actually be realized within the architecture. For

example, ASR 8: your description says you need to support seamless ML Model updates, and your architectural impact boils down to the same message, the need to support seamless ML model updates. Explain *how* these seamless updates can be achieved within the architecture.

- ASR 14 needs full revision - it is not fully clear what you wanted to say here. As per the case description, the PMS is a totally standalone system, why would it be dependent on the HIS infrastructure in the first place? Repeating this in an ASR does not bring additional value.
- The overall quality of your QAS is good, but here too, they could benefit from additional specificity where possible. Some responses remain too vague, for example in QAS 1: “the system detects...”, which system exactly?
- QAS 2: the response measures could be clearer defined - now it seems to be a complicated way of repeating ‘there should not be errors’ and ‘the HL7 FHIR spec should be followed’, but how would you measure this? When an error is found, you mention it should be retried - would you be measuring errors that do not succeed after retrying, or is an error counted even if a retry succeeds?
- QAS 3: the responses given seem somewhat vague and high-level. Be more precise in which kind of instances/nodes the system should provision, are you talking about storage or processing infrastructure here? Are there any other responses besides what you list here?

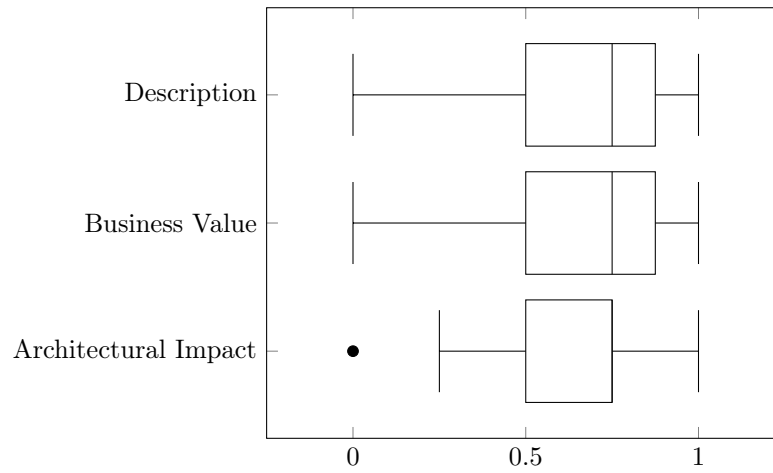
1 Feedback on the ASRs

This section first provides feedback on the quality of the ASRs.

1.1 Breadth and coverage

The coverage of your ASRs is very good. You have a lot of variation in different qualities in your report to obtain a diverse set of ASRs.

1.2 Quality of your descriptions



2 Quality Attribute Scenarios

